Turnage, Kevin D. (IHS/PHX)

From: Benjamin, Brandi (IHS/PHX/PIMC)
Sent: Wednesday, February 8, 2023 8:18 AM
To: Turnage, Kevin D. (IHS/PHX)
Subject: RE: What are we doing and not doing?

Kevin, I think this email is great, it reflects your dedication to our mission and as a group that it should always be PATIENTS FIRST! You have the ability to do great things with this clinic!
Brandi

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Tuesday, February 7, 2023 9:37 PM
To: Andreasen, Gloria (IHS/PHX) [C] <Gloria.Andreasen@ihs.gov>; Begay, Lynette C. (IHS/PHX) <Lynette.Begay@ihs.gov>; Buck, Christine (IHS/PHX) <Christine.Buck@ihs.gov>; Burbank, Nedra (IHS/PHX) <Nedra.Burbank@ihs.gov>; Clark, Roxanne J. (IHS/PHX) <Roxanne.Clark2@ihs.gov>; Earl, Erica (IHS/PHX) [C] <Erica.Earl@ihs.gov>; Gbemu, Jeritha E. (IHS/PHX) <Jeritha.Gbemu@ihs.gov>; Jensen, Brenda (IHS/PHX) <Brenda.Jensen@ihs.gov>; Keene, Artisha (IHS/PHX) <Artisha.Keene@ihs.gov>; Moreno, Michelle (IHS/PHX) <Michelle.Moreno@ihs.gov>; San Nicolas, Sharon D. (IHS/PHX) <Sharon.SanNicolas@ihs.gov>; Schippers, Jennifer (IHS/PHX) <Jennifer.Schippers@ihs.gov>; Shirley, Sharlene (IHS/PHX) <Sharlene.Shirley@ihs.gov>; Smith, Lori M. (IHS/PHX) <Lori.Smith@ihs.gov>; Starr, Karyn (IHS/PHX) <Karyn.Starr@ihs.gov>; Gomez, Viridiana (IHS/PHX/PIMC) [C] <Viridiana.Gomez@ihs.gov>; Frost, Arlene (IHS/PHX/PIMC) [C] <Arlene.Frost@ihs.gov>; Barr, Brenda (IHS/PHX) <Brenda.Barr@ihs.gov>; Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>; Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; John, Gladys (IHS/PHX) <Gladys.John@ihs.gov>; Semallie, Melissa (IHS/PHX) <Melissa.Semallie@ihs.gov>; Tessay, Vanessa R. (IHS/PHX) <Vanessa.Tessay@ihs.gov>
Cc: Benjamin, Brandi (IHS/PHX/PIMC) <Brandi.Benjamin@ihs.gov>; Baek, Aimee (IHS/PHX) <Aimee.Baek@ihs.gov>; Ruiz, Sara L (IHS/PHX) <Sara.Ruiz@ihs.gov>; Goswami, Yamini (IHS/PHX) <Yamini.Goswami@ihs.gov>
Subject: What are we doing and not doing?
Importance: High

Thank you for all your hard work during this respiratory/flu/COVID season and I want to tell you all how much I appreciate everyone focusing on putting our patients first in all decisions. I also want to let you all know how much I appreciate the texts and emails I received while out on leave they made a huge difference in my return as soon as I did.

There have been a few issues that have been noted and/or that I have been made aware that I want to address collectively so that everyone is aware of expectations and requirements.

1. A lot of diligent work went into improving our access of medical records with other facilities and their Health Information Exchanges. All CCC's, CMA's and MSA's are expected to have and maintain access to all portal exchanges (Banner, Dignity, Honor Health, Valleywise, Mayo and SimonMed). If you do not currently have access to any of these please let me know so we can get them established for you. I am still seeing faxes sent to these facilities for records instead of logging in and downloading the records. This causes delays and fragmentation in patient care and actually causes more work in the long run. If there are other facilities that you feel the patients and clinic may benefit from gaining access to please let me know so I can try and facilitate access.
2. In making rounds with administrators and others in the clinic it has become extremely obvious that the use of cell phones and ear buds has become an issue. There have been several instances of people sitting at their workstation with movies and other videos playing, not just once but on a regular sometimes daily basis. I have attached a current copy of the "Cell Phones and Electronic Devices" policy, please ensure that you read this policy. The policy is clear in that "Ear buds shall not be in use in areas where internal or external customers may need to access services. Earbuds shall not be used in public areas." This includes team rooms and both earbuds

should only be used for training/web calls etc. all other times then both ears should not be obstructed or have buds in place. If music is being played via a speaker please be mindful of the requirements if others object to the music. I will be enforcing this policy especially as it relates to playing/watching movies and videos during work hours and not on breaks. I understand completely the need to have your personal cell phone at work and the need to communicate with family. I ask that you be mindful and not to abuse the privilege so that it does not cause further scrutiny of cell phone usage for everyone.

3. I am looking at our nurse only visits/clinic and looking for ideas to improve/streamline the process including scheduling. If you have any suggestions or ideas please get them to me so we can look at all options for improvement. If you have suggestions or ideas about this or any of our other processes I welcome those as well.
4. There are many changes coming to the clinic to ensure that we are meeting the Primary Care Medical Home Model, that patients are always put first and that we practice first contact care that is person-focused, comprehensive, coordinated and continuous. If we can do something for the patient lets do it and not just pass the buck on to someone else. The care will be person-centered, team-based, community-aligned, and designed to achieve better health, and better care for our patients. Primary care needs to be there for our patients when they need it whether it be breathlessness in asthma, foot wounds in diabetes, and suicidality in depression, as much as it needs to be ready with sutures, splints, and ordering imaging and medication. We need true first-contact care and It is time healthcare got back to its core task of attempting to relieve anxiety of the unknown, pain and suffering for patients and their loved ones. Change is always difficult but if it is for the betterment of our patients I ask that everyone try to embrace the changes. However, if you have concerns let me and leadership know. Remember if a change is made that does not work we can always go back but we must truly try and change to know if a change fails because of a process or if it fails because we do not genuinely try and make change work.

Finally, I am proud to say that I am part of the Primary Care Team at PIMC because we work hard daily to provide our patients with the best care possible. It is truly an honor to say I am part of this team and I know we can do great things for the betterment of all our patients not only in Phoenix but across all of IHS by setting the example and paving the way for others.

Hon. Kevin D. Turnage RN, MSN Ed. AMB-BC, NHDP- BC, NPD-BC
Supervisory Clinical Nurse PCMC and Employee Health , COR II,
Phoenix Indian Medical Center
602-263-1574
Cell: 602-647-9126

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